جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : جوفانه عادل موريس شحاته
المحافظة : القاهره **الحي / المدينة :** مصر القديمه **العنوان :** القاهره
رقم البطاقة : 28103070102608 **رقم التليفون :** 01125827844 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 07-03-1981 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمده 6 شهور [14163] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنه و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: جوفانه عادل موريس شحاته
National ID: 28103070102608

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Goserelin (Zoladex) subcutaneous**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



National ID – Front
[[DOCTYPE:ID_FRONT]]

National ID – Back

[[DOCTYPE:ID_BACK]]

Name:	**جوفانه عادل موريس شحاته**
MR. :	**13249**

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

45 years, HTN, Psoriasis SM: RT breast multifocal pathologically enhancing malignant lesion Rt breast mass, biopsy: IDC g3, ER 8/8 PR 8/8 Her2: 2+ (pending SISH) 25/8 CT CAP: Neoplastic RT breast Mass + benign looking Rt axillary LN, simple left ovarian cyst, mild pericardial effusion, subpleural reticulations Brain MRI: diffuse subcortical leukoencephalopathy For Bone Scan + neurology consultation + echo

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : **Khaled Abd Elaziz Mohamed Kamal**

يعتمد ,

MRN:	10126283088		
Patient Name:	Gouvana ADEL MOREES SHEHATA	D.O.B:	07/03/1981
IC No.:	28103070102608	Sex:	Female
Location:	External Clinic		

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2607210100	**Collection Date**	21/07/2026 10:14 AM
Accession No	SUR-26-2554	**Received Date**	21/07/2026 10:14 AM
Sample Type	Tissue	**Doctor**	shrouk mahmoud mohamed Mohamed

Clinical Information
Right breast lesion at 9-10 O'clock (BIRADS 5).

Gross Picture
Multiple grayish tissue cores 0.6-1.3 cm long, totally submitted.

Microscopic Picture
A case of invasive duct carcinoma, grade 3. No DCIS component. There is evidence of lymphovascular emboli. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:
Invasive duct carcinoma, NST, grade 3.

Laterality
Right

Specimen type:
U/S guided core biopsy.

Location
9-10 O'clock

Lesion Size

Invasive tumor size/s (mm):

Total extent of lesion (mm):

Multiple Lesions

Histological grade

Mitosis score (1-3):
3

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Tubular formation score (1-3):
3

Nuclear grade (1-3):
3

Histological grade (1-3):
3

Peritumoural lymphovascular invasion
Present

DCIS in specimen

Absent

Diagnosis

Right breast lesion, U/S guided core biopsy:
Invasive duct carcinoma, NST, grade 3.

Comments
ER, PR & HER-2 are available(upon request).

Biopsy reporting category
B5b= Invasive cancer

Signature
Dr. Ahmed Ali. MD.

Co-signed by:
Dr. Hazem Abdelazim, MD, IFCAP

Reported and electronically endorsed by
Hazem Abdulazim Sobhy Abo Ismael

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Laterality

Signature
Dr.Mahitab Eltohamy ,MD

Co-Signed by:

Reported and electronically endorsed by
Mahitab Mohamed Eltohamy Mohamed

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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مستشفى بهية الشيخ زايد
Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

MRN:	10126283088	D.O.B:	07/03/1981
Patient Name:	Gouvana ADEL MOREES SHEHATA	Sex:	Female
IC No.:	28103070102608		
Location:	External Clinic		

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id	2608020098	**Collection Date**	02/08/2026 10:56 AM
Accession No	IMM-26-3190	**Received Date**	02/08/2026 10:56 AM
Sample Type	Paraffin Block	**Doctor**	Mahmoud sayed Abdelmohsen Meshref

Immunohistochemical Report

pathology number
2554/26

Clinical Data
Rt. breast cancer

Gross Picture :
Referred block submitted for HER 2 immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within <=10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within >10% of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in >10% of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in >10 % of tumor cells.

(Laterality) breast Cancer:
Right

Her2 Result
Equivocal [score 2+]. Recommended For SISH

Comment :
**SISH test for HER-2 gene copy number is available on request.

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مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد
Immunohistochemistry
[[DOCTYPE:IHC]]

MRN:	10126283088		
Patient Name:	Gouvana ADEL MOREES SHEHATA	D.O.B:	07/03/1981
IC No.:	28103070102608	Sex:	Female
Location:	External Clinic		

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	2608020096	**Collection Date**	02/08/2026 10:56 AM
Accession No	IMM-26-3191	**Received Date**	02/08/2026 10:56 AM
Sample Type	Paraffin Block	**Doctor**	Mahmoud sayed Abdelmohsen Meshref

Immunohistochemical Report

pathology number
2554/26

Clinical Data
Rt. breast cancer

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%–100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.

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مستشفى بهية الشيخ زايد
١٠ الحي الأول ، الشيخ زايد أمام كلية الهندسة
Immunohistochemistry
[[DOCTYPE:IHC]]

MRN:	10126283088	D.O.B:	07/03/1981
Patient Name:	Gouvana ADEL MOREES SHEHATA		
IC No.:	28103070102608	Sex:	Female
Location:	External Clinic		

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2607210100	**Collection Date**	21/07/2026 10:14 AM
Accession No	SUR-26-2554	**Received Date**	21/07/2026 10:14 AM
Sample Type	Tissue	**Doctor**	shrouk mahmoud mohamed Mohamed

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Tumour type:
Invasive duct carcinoma, NST, grade 3.

Laterality
Right

Specimen type:
U/S guided core biopsy.

Location
9-10 O'clock

Lesion Size

Invasive tumor size/s (mm):

Total extent of lesion (mm):

Multiple Lesions

Histological grade

Mitosis score (1-3):
3

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مستشفى بهية الشيخ زايد
-1 الحي الأول - الشيخ زايد - الهندسة
Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

Right

Diagnosis :

ER : positive, strong nuclear reaction in 99 % of tumor cells (8/8).
PR :positive, strong nuclear reaction in 100% of tumor cells (8/8).

Signature

Dr.Mahitab Eltohamy ,MD

Reported and electronically endorsed by

Mahitab Mohamed Eltohamy Mohamed

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

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مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد أكتوبر الهندسية
Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Tubular formation score (1-3):
3

Nuclear grade (1-3):
3

Histological grade (1-3):
3

Peritumoural lymphovascular invasion
Present

DCIS in specimen

Absent

Diagnosis

Right breast lesion, U/S guided core biopsy:
Invasive duct carcinoma, NST, grade 3.

Comments
ER, PR & HER-2 are available(upon request).

Biopsy reporting category
B5b= Invasive cancer

Signature
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مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد - أكتوبر الهندسة
Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

PATIENT NAME: Gouvana ADEL MOREES SHEHATA **GENDER:** Female **DATE :** 20-Jul-2026 12:39 PM	**PATIENT ID:** 10126283088 **AGE:** 45 y **ORDER:** 3963046,3963047

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:

- Right breast palpable lump since two months.

TECHNIQUE:

- CC and MLO views.
- Complementary ultrasound.

REPORT:

- Heterogenous dense breasts (ACR C).

Right Breast:

- UOQ two ill defined spiculated dense lesions measuring roughly about 4.4x4.5 cm, 3 cm away from the nipple (showing internal pleomorphic calcifications) & 3x4.1 cm, 4 cm away from the nipple. (measured in CC view)
- Complementary US revealed UOQ multiple ill defined spiculated hypoechoic lesions are seen at:

- 11-1:00 measuring about 2x3.6 cm, 0.9 cm away from the overlying skin. Prominent ducts are seen related to the medial aspect of the lesion, ? intraductal extension.
- 9-10:00 measuring 2x4.7 cm, 0.7 cm away from the overlying skin.
- Two small satellite lesions are seen at 11:00 & 10:00 measuring about 0.9x1.2 cm (0.9 cm away from 10:00 index lesion) & 0.5x0.6 cm (0.3 cm away from 12:00 index lesion) respectively.

- Normal skin thickness and contour.
- Normal nipple areola complex.
- No detected pathological axillary or supraclavicular lymph nodes.

Left Breast:

- No spiculated lesions or grouped microcalcifications.
- Complementary Ultrasound showed no detected solid or cystic lesions.
- Normal skin thickness and contour.
- Normal nipple areola complex.

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Mammogram
[[DOCTYPE:MAMMOGRAM]]

- Nonspecific axillary lymph nodes.

OPINION:

- Right breast UOQ multiple suspicious lesions, for histopathological assessment (BIRADS 5).
- Left breast adenosis (BIRADS 2).

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Lamiaa Abdallah,MSc **20-Jul-2026 02:11 PM**

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٤ شارع متفرع من شارع الهرم بجوار نادي الجزيرة
مستشفى بهية للشيخ زايد
الشيخ زايد أمام كلية الهندسة
Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient's Name:	**Jovanna Adel Mouris Shehata**
Date:	**2026-09-02**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

***MRI OF THE BRAIN* WITH CONTRAST :**

Axial T1, T2 ,FLAIR,
Coronal T2
Sagittal T1
Postcontrast T1

Findings:

- Few tiny bilateral cerebral periventricular foci of abnormal MR signal are seen displaying low signal on T1 that turned bright on T2 & FLAIR with no contrast uptake. No mass effect or perifocal oedema.
- Normal size and configuration of the ventricular system. Bright T2 & FLAIR signal are seen framing the periventricular white matter.
- No midline shift or deformity
- Prominent cortical sulci and extra axial CSF spaces.
- No extra axial collections or intra axial masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Prominent peri-optic CSF Spaces bilaterally.

OPINION:

Few tiny bilateral cerebral lacunar infarctions.
Diffuse subcortical arteriolo-sclerotic leukoencephalopathy.
Prominent peri-optic CSF spaces bilaterally for clinical correlation.

Dr.sherif Wagdy

M.R.I
[[DOCTYPE:MRI]]

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصرية صحة مصر

10/12/2026

MDT FORM

Name:	جوقته عادل مرريس شحته	**MRN:**	13249
Age:	46 Y	**ID:**	28103070102608
Menopausal Sta	pre	**Phone:**	01273357774
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:	HTN	**Reason Of MDT:**	new

Clinical Data:

BX: rt IDC G 2

ER: 8/8 **PR:** 8/8 **HER2:** 2+ not an **KI67:**

Sonomammography:

sonomamo: ACR C

rt breast UOQ multiple susp lesions , no pathologiacl LNs (BIRADS 5)

LT breast (BIRADS 2)

CESM : rt 12:00 and UOQ tow masses with anterior intraductal extension not reaching the nipple 40x25 mm , 36x 33 mm

rt multifoval

Mets Workup:

CT : Neoplastic lesions in the right breast and variable sized right axillary lymphadenopathy .

• Relatively thickened left adrenal gland, likely to be a hyperplasia rather than a metastasis.

. Subpleural reticulations in the apical and posterior segments of both lower pulmonary lobes and linear atelectatic bands in the middle lobe and in the lingula. Mild pericardial effusion.

Hepatosplenomegaly with no focal lesions.

A simple left ovarian cyst. bone scan no mets

History:

clinically : medium size breast with palpable 2 masses at 12:00 and UOQ with mild skin tethering and mild edema

Photo sent

for possibility of NACT then rt BCS ???

Recommendations:

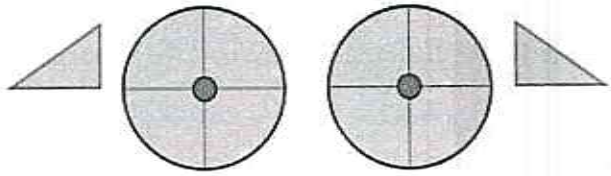
MDT Outcome & Plan

In view of having cT2 bifocal disease, not amenable to conservation....for :

1.Clip insertion of both masses

2.Portacath insertion then start NACT AC x 4 followed by Taxotere x4, with Zoladex then re-do CESM and consider Rt WLE & SLNB accordingly.

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د شيماء لاشين	د امنيه عيسوي				

12/10/2026 12/10/2026

[[DOCTYPE:SONOGRAPHY]]
Sonography

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Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر-معمل الباثولوجي
جوستاف روسي

Patient Name	جوفانه عادل موريس	**Case No.**	**360215**
Sex/Age	**Female 45 Y**	**Patient No.**	**26264**
Registered Date	**24/8/2026 12:42:30 PM**	**Referred By Dr.**	**Out Patient**
Validation Date	**27/9/2026 09:58:55 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**01/10/2026 11:30:59 AM**	**Pathology No.**	**SR-972/26**

Breast Slide Revision

Clinical Data: Right breast 9-10 o'clock lesion (BIRADS 5).

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:
- One referred block labelled by (2554-26). Together with, four referred slides including 1 (H&E) stained slide and 3 immunostained slides for (ER, PR & Her2).

Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for; SISH.

Microscopic Examination:
- **Histologic type:** Invasive breast carcinoma, NST.
- **Histologic grade: 2** (Differentiation 2, pleomorphism 2, mitosis 2)
- **DCIS:** 5%.
- **Microcalcification:** Absent.
- **Angioinvasion:** Present.
- **Perineural invasion:** Absent.

*** IPT:**
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified. (Average Her2 copy No.: (4.55), Her2/CEP17 ratio: (1.2)). Done in our lab and counted twice.

Conclusion:
Right breast mass, Tru-cut biopsy:
- Invasive breast carcinoma, NST, Grade 2.
- 5% Ductal carcinoma in situ.
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group IV. (Average Her2 copy No.: (4.55), Her2/CEP17 ratio: (1.2)).

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Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	جوفانه عادل موريس	**Case No.**	**360215**
Sex/Age	**Female 45 Y**	**Patient No.**	**26264**
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Validation Date	**27/9/2026 09:58:55 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**01/10/2026 11:30:59 AM**	**Pathology No.**	**SR-972/26**

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

** ***Scoring system: "Her2"*** Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.
ISH groups:
The ratio is calculated and the results is interpreted as:
• Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
• Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
• Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
• Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
• Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Prof. Dr. Maha Guimei.
Prof. Dr. Rana El Saeed.
Dr. Walaa Ibrahim Ghonaim.

[[DOCTYPE:BONE_SCAN]]
Bone Scan

Name : Jovanna Adel Morris Shehata
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 03-10-2026 07:29
Medical No. : 475089

NILE DIAGNOSTIC CENTER

Tc-99m MDP SKELETAL SCINTIGRAPHY

History

45 year-old female patient, with history of right breast cancer and referred for skeletal survey.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Warm tracer uptake is noticed at left sterno-clavicular articulation and left side of LV5.
- Degenerative changes are noticed at different levels of the spine.
- Osteoarthritic features are noticed around big joints.
- Rest of the scanned skeleton s unremarkable.

IMPRESSION:-

- Benign looking lesions at the left sterno-clavicular region and left side of LV5, for follow up/ CT correlation.
- Rest of the surveyed skeleton is unremarkable.

Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine

GUSTAVE ROUSSY INTERNATIONAL

جوستاف روسي انترناشونال مصر

ECHO REPORT

NAME : جوفانه عادل DATE: 1 /10/2026

ID NUMBER : / SEX : ♀ AGE: 45

EF	62 %	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO.R	2.8	3-3.7cm
LVESD		2-4cm	LA	4.2	2-4cm
FS		25-45%	RV		0.7-2.3com(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions — normal

LV Wall Thickness — normal

LV Diastolic Function — DD grade I

SWMA — no SWMA

LA and Ao Root Dimensions — Dilated LA

Mitral Valve — Mild TR

Aortic Valve — normal

Tricuspid Valve — Mild TR

Pulmonary Valve — normal

Right Side and Systolic Function — normal

Intracardiac Masses and Effusion — -

DR. د/ مناحل